

Importance of Medication Reconciliation Done by Pharmacy Technicians

Introduction

Medication reconciliation (med-rec) is an essential process within hospitals, designed to guarantee precise communication of all medication information throughout a patient's treatment. Pharmacy technicians play a key part by assisting pharmacists, nurses, and physicians with collecting, verifying, and recording medication histories. "The Joint Commission defines the term "medication reconciliation" as "the process of comparing the medications a patient should be using (and is actually using) to the new medications that are ordered for the patient" in an effort to resolve any discrepancies in therapy."⁴ This document describes standard procedures and policies for pharmacy technicians managing med-rec in hospitals, because research shows that up to 67% of patients have at least one error in their documented medication history when admitted to hospital, and between 30% and 80% experience discrepancies between medications prescribed in hospital and those they were previously taking at home.³ Every year in the United States, medication mistakes during patient discharge affect 1.5 million people, resulting in a substantial number of deaths and costing about \$3.5 billion to healthcare system.³

Objectives

A study found that pharmacy technicians and students enhance medication history accuracy in hospitals, with their group showing fewer clinically significant errors per medication compared to nurses (0.03 vs 0.09)⁵ therefore the main objectives for this program is to:

- Ensure patient safety by minimizing medication errors during transitions of care.
- Accurately obtain and document comprehensive medication histories.
- Support multidisciplinary teams in providing optimal pharmaceutical care.

Scope of Practice

A 2010 study evaluated the accuracy and effectiveness of pharmacy technicians compared to nurses in conducting medication reconciliation tasks, including obtaining patient history, verifying information with pharmacies, and documentation. Findings indicated that pharmacy technicians produced more accurate lists and achieved higher completion rates. Additionally, technicians demonstrated superior communication with patients,

effectively addressing issues related to non-adherence. Another investigation showed that integrating pharmacy technicians into medication reconciliation processes led to an 82% reduction in error rates.¹ A 2017 study further revealed that medication histories obtained by pharmacy technicians in the emergency department were significantly more accurate than those recorded by nurses; every medication history documented by nurses contained at least one error, whereas technicians' histories contained errors in 36% of cases. The overall accuracy rate was 14% for nurses and 94.4% for pharmacy technicians. Utilizing pharmacy technicians in this capacity enables pharmacists to focus on other clinical responsibilities. Pharmacy technicians and interns may perform medication reconciliation under the supervision of a licensed pharmacist, where they are tasked with collecting medication information, and the final medication list must be reviewed and verified by the pharmacist prior to clinical decision-making. The study concluded that employing pharmacy technicians for medication history documentation is more cost-effective than hiring additional nurses or pharmacists.²

Procedure for Medication Reconciliation

Prepare yourself before entering the room for an interview. Review the admission notes to learn the patient's medical history and the reason for their hospitalization (the chief complaint). Consider whether any of the patient's medications might have led to their current hospital admission. Check for recorded allergies and note any relevant reactions. Also, see if there is a current medication list available in the chart.⁶

Step-by-Step Process:

1. Get permission to go in the room from the patient and then Introduce yourself.
2. Get patient Consent to continue and (make your own judgment too if patient is in good condition to talk about his/her medications) Confirm the patient's identity using at least two identifiers (e.g., name and date of birth).
3. Explain the purpose of medication reconciliation and obtain verbal consent.
4. Obtain a comprehensive medication history by inquiring about all prescription medications you take daily, including those prescribed by your primary care physician or any specialists. (make sure to get the dose, directions, formulation, and route) and how patient takes (as prescribed or doing something different?) **Ask open- ended questions?**
5. You may Interview patient's family members, or caregivers or the person take care of patient's medications at home.
6. Review available documentation, such as previous medical records, prescription bottles, medication lists, and pharmacy profiles.

7. Ask about prescription medications, over-the-counter drugs, herbal supplements, vitamins, and as-needed medications.
8. What drug or other allergies patient have and what adverse reaction have occurred previously.
9. Get information about patient preferred outpatient pharmacy where hospitalists may send the prescription at the time of discharge.
10. Verify all the Information obtained.
11. Clarify any discrepancies or uncertainties with the patient or caregiver.
12. Contact outpatient pharmacies or providers as needed for verification.
13. Record all current medications, including drug name, dose, route, frequency, and indication, in the designated hospital system (electronic health record or paper chart).
14. Note medication allergies and adverse drug reactions.
15. Report findings and potential discrepancies to the supervising pharmacist promptly.
16. **Ensure the pharmacist reviews and validates the medication history prior to physician orders or clinical interventions.**

Pharmacy technicians should assess whether any barriers exist that may hinder the patient's adherence to their medication regimen. Targeted questions help create effective discharge plans, encourage ongoing medication use, and lower readmission rates. Appropriate questions may include:

- Do you sometimes forget to take your medicine?
- Tell me how you take this medicine at home.
- What medicines have you started or stopped taking without your doctor knowing?
- How do you usually pay for your medicines?
- What barriers do you have to keep the adherence to your current medications?

Policies

- Pharmacy technicians must complete hospital-approved training in medication reconciliation procedures.
- All collected medication histories are subject to pharmacist review and approval before use in clinical decision-making.
- Confidentiality of patient information must be maintained in accordance with HIPAA and hospital privacy policies.
- Any discovered medication discrepancies or potential adverse drug events must be reported immediately to the pharmacy and clinical teams.

- Documentation should be timely, accurate, and legible.
- Continuous quality improvement activities, such as audits and feedback, should be performed to ensure high standards in medication reconciliation.

Best Practices

- Use standardized forms or electronic systems for documentation.
- Engage patients and caregivers in the process to improve accuracy.
- Regularly update training and policies based on regulatory changes and hospital needs.
- Collaborate with other healthcare professionals to resolve medication discrepancies.

Conclusion

Pharmacy technicians are vital contributors to safe and effective medication reconciliation in hospitals. By following these procedures and policies, technicians help reduce medication errors, improve patient outcomes, and support the healthcare team in delivering high-quality care.

APA Citations and Web Link:

1. Patel, S. (2024, July 26). *Medication reconciliation: A unique opportunity for Pharmacy technicians*. freeCE. <https://www.freece.com/blog/medication-reconciliation-a-unique-opportunity-for-pharmacy-technicians/>
<https://www.freece.com/blog/medication-reconciliation-a-unique-opportunity-for-pharmacy-technicians/>
2. Markovic, M., Mathis, A. S., Ghin, H. L., Gardiner, M., & Fahim, G. (2017). A Comparison of Medication Histories Obtained by a Pharmacy Technician Versus Nurses in the Emergency Department. *P & T: a peer-reviewed journal for formulary management*, 42(1), 41–46.
<https://pmc.ncbi.nlm.nih.gov/articles/PMC5215278/>
3. *The High5s Project -Standard Operating Protocol for Medication Reconciliation Standard Operating Protocol Assuring Medication Accuracy at Transitions in Care*. (n.d.). https://cdn.who.int/media/docs/default-source/patient-safety/high5s/h5s-sop.pdf?sfvrsn=594d8e49_4
4. *National Patient Safety Goals® Effective January 2025 for the Hospital Program Goal 1 Improve the accuracy of patient identification*. (n.d.). <https://digitalassets.jointcommission.org/api/public/content/9be383450fc941df806b76c5fbdd9ae6?v=3c600c3a>
5. Bowman C, McKenna J, Schneider P, Barnes B. Comparison of Medication History Accuracy Between Nurses and Pharmacy Personnel. *Journal of Pharmacy Practice*. 2017;32(1):62-67. doi:10.1177/0897190017739982
<https://journals.sagepub.com/doi/10.1177/0897190017739982>
6. <https://www.ashp.org/-/media/assets/pharmacy-technician/docs/EdandTrainMedRecOverviewUICChoye.pdf>

